



CIN No. U85110WB2005NPL104948

Institute of Neurosciences Kolkata

Twinned With the Regional Neurosciences Centre, Newcastle upon Tyne, UK

Affiliates : Ram Gopal Chamarla Medical Research Centre

Santanu Sengupta Memorial Stroke Centre

Jain Misrilal Padmawati Foundation Medical Rehabilitation Centre



INK / AAC / FM / 36

Patient Name : Mrs. SUBHRA DEY

Age/Sex : 66Yrs/Female

PRN/Visit : 115123/OP-12

Visit Date : 05/03/2026 11:57

Consulting Dr : Dr. Purba Basu

Department : Neurology

Referred By :

OP CLINICAL RECORD

Diagnosis

:Late onset ataxia--possibly MSA-C Multi-System Atrophy; Cerebellar type) (symptomatic since 2010)/ SCA 40

[Mother had similar gait imbalance and recurrent falls----SCA gene profile---negative]

Patient has predominantly ataxic syndrome with more than 10 years history of disease. Mother had similar illness. She had been investigated with SCA profile which has come as negative. Tentatively MSA was considered. The slow progression of disease might go against MSA but some patients with MSA may have reasonably benign course.

Present Complaints

Overall worsened than before---worsened over last one month

Gait imbalance with falls

Unable to walk without significant support

Slurred speech all the time

On and off hallucination---both auditory and visual

Impaired memory---worsening

Incoordination of fingers and loss of dexterity

Constipation---worsened than before

Sleep worsened

Urine C/S negative

History

57 year old lady is having unsteady gait since 2010. Very slow progress of the symptoms. Intermittent tremors in both hands. Numbness and uncomfortable sensation in both legs since last 2-3 months. Gait imbalance---has tendency to fall. Occasional retrograde walking. Difficulty in getting up from ground. Urinary urge incontinence. Sleep normal. Memory normal.

No Diabetes Mellitus. Known hypertensive. Mother had developed ataxia during end years of her life.

O/E---Eye movements normal. Speech hypomimic mildly dysarthric. No tremor, no rigidity, no bradykinesia. Walks with broad based gait and normal arm swing. Unable to walk on straight line. Tandem impaired. Pull test normal. DTRS brisk in both upper and lower limbs. head tremor present FNT- positive bilaterally

MRI brain (2016)---Multiple chronic ischaemic foci with diffuse cortical atrophy. No significant cerebellar atrophy.

MRI Screening cervical spine (2016)---Degenerative disc disease. No obvious compression, no signal changes.

Investigations Advised:

SCA gene profile-negative

8/12/2023- Clinical Exome sequencing - uncertain significance variant in the CCD88C gene probably causative of the reported

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phenotype - associated with probable SCA 40

MRI brain (Oct, 2025) White matter ischaemic changes. Cerebral atrophy. Left thalamic, frontal and parietal old microbleed

Has LOC in Sep, 205-----felt weakness of left side and was drowsy for half an hour. --systolic BP was < 100 mm of Hg
Had again had LOC for 15 seconds next week.
MRI brain was non contributory

Medicines Advised

Ciplar LA (Propanolol) 40mg 1 - 0 - 0

(Levodopa, Benserazide) Madopar 250mg 1 - -1--1--1

(7am - 11am - 3pm - 7pm)

(Quetiapine) Qutan 25mg
0--0--1 for 7 days followed by
1/2--0--1 continue

(Paraffin) Syr Cremaffin Plus 0--0--15ml ---sos

(Rivastigmine) Rivamer 1.5mg 1--0--1

(Methylcobal) Inj Rejunex 500mcg IM once a week for 8 weeks

Other medicines continuing(to adjust with treating specialist)

TLM 40 once a day
Dytor 10mg 1--0--0
Atorlip 20 once a day
Ecosprin 150 once a day

Gait and Balance exercise to continue

Speech therapy

Follow up after 4 months

Quadriceps strengthening,
Feeling cycle

Investigation Advised

CBC, ESR, CRP, Na, K

Chh

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Page 2 of 3